

1. Administrative & Study Identification

Full Study Title: A Phase 1b/2 Open-label Study Evaluating Different MK-6070 and Ifinatamab Deruxtecan (MK-2400)-Based Regimens in First-line Extensive Stage Small Cell Lung Cancer
Short Title/Acronym: MK-6070-003 **Protocol Number:** MK-6070-003 **NCT Number:** NCT07227597 **Posting ID:** 369951154487414396 **Trial Status:** Not Yet Recruiting **Sponsor Name:** Merck Sharp & Dohme (MSD) / Merck **Investigational Product:** Gocatamig (MK-6070, a T-cell engager) and Ifinatamab Deruxtecan (I-DXd / MK-2400, an antibody-drug conjugate) **Phase of Development:** Phase 1, Phase 2 (Phase 1b/2) **Medical Monitor:** Medical Director, Merck Sharp & Dohme (MSD)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and early clinical signals of different treatment regimens involving gocatamig (MK-6070) and ifinatamab deruxtecan (I-DXd) as first-line therapy for extensive-stage small cell lung cancer (ES-SCLC). **Secondary Objectives:** To evaluate Disease Control Rate (DCR), Duration of Response (DOR), and Progression-Free Survival (PFS).

2.2 Background & Rationale Small cell lung cancer (SCLC) is an aggressive neuroendocrine carcinoma with a poor prognosis. While the standard of care (SOC) using chemo-immunotherapy often yields initial responses, this is unfortunately followed by rapid disease recurrence in most patients. Gocatamig is a trispecific T-cell engager therapy targeting DLL3, which is highly expressed in SCLC. Ifinatamab deruxtecan (I-DXd) is an antibody-drug conjugate (ADC) designed to destroy cancer cells. Researchers are conducting this study to see if combining gocatamig and I-DXd— with or without standard chemotherapy/immunotherapy—can provide a more durable and effective treatment for people with ES-SCLC.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active-controlled (Standard of Care arm) **Blinding:** Open-label **Randomization:** Randomized (In Part B, participants are allocated to Arm 1 per investigator discretion and randomized to Arms 2, 3, and 4)

3.2 Planned Enrollment & Duration Target \$\$\$: Approximately 170 participants **Number of Sites:** Global multi-center **Estimated Study**

Start: 29-Jan-2026 **Estimated Primary Completion:** 29-Nov-2030
(Study End Date)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adults with histologically or cytologically confirmed Extensive-Stage Small Cell Lung Cancer (ES-SCLC). 2. Participants who are treatment-naïve (did not receive prior systemic treatment for ES-SCLC) for specific induction arms. 3. Participants receiving gocatamig + I-DXd in maintenance only must have completed 3 to 4 cycles of standard of care (SOC) induction platinum + etoposide chemotherapy with concurrent approved anti-PD-1/L1 treatment without radiological disease progression per RECIST 1.1.

4.2 Exclusion Criteria 1. Documented disease progression prior to the maintenance phase (for applicable maintenance-only cohorts). 2. Has history of (noninfectious) pneumonitis/interstitial lung disease (ILD) that required steroids, current pneumonitis/ILD, or suspected pneumonitis/ILD. 3. History of clinically significant intracranial bleeding, spinal cord bleeding, or active neurologic paraneoplastic syndrome. 4. Other protocol-defined exclusion criteria, such as known active central nervous system (CNS) metastases, carcinomatous meningitis, or prior exposure to anti-PD-1/L1 therapy (unless permitted in specific study phases).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Dosing regimens depend on the assigned cohort and study part (Part A or Part B):

* **Investigational Arms:** Gocatamig + I-DXd during induction and maintenance; OR Gocatamig + I-DXd in induction followed by Gocatamig + Atezolizumab in maintenance. * **Maintenance-Only Arm:** Gocatamig + I-DXd during maintenance following SOC induction. * **Active Comparator Arm:** SOC (carboplatin + etoposide [ATC Code: L01CB01; Trade names: Etopophos, Vepesid, Toposar] + atezolizumab) in induction followed by atezolizumab maintenance.

Route of Administration: Intravenous (IV) infusion for biologic therapies and standard chemotherapies. **Duration of Treatment:** Continued until documented disease progression or meeting other study discontinuation criteria.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for ES-SCLC symptoms and chemotherapy-induced toxicities. **Prohibited:** Concurrent investigational anti-cancer therapies and live vaccines during treatment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Imaging, Baseline Labs
Baseline	Day 0	Randomization/Allocation, First Dose of Study Intervention
Interim	Varies	Safety Labs, Tumor Imaging (e.g., RECIST 1.1 assessment), Efficacy Evaluations
End of Study	Until Progression	Final Safety Labs, Tumor Assessment, Exit Interview, Follow-up for Survival

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints

- 1. Adverse Events (AE):** Number of Participants Who Experience an Adverse Event (defined as any untoward medical occurrence temporally associated with the use of study intervention).
- 2. Dose-Limiting Toxicities (DLTs):** Number of Participants Who Experience One or More DLTs (defined as any drug-related AE observed during the DLT evaluation period of up to 21 days).
- 3. Discontinuation Due to AE:** Number of Participants Who Discontinue Study Intervention Due to an AE.

7.2 Secondary Endpoints

- 1. Disease Control Rate (DCR)**
- 2. Duration of Response (DOR)**
- 3. Progression-Free Survival (PFS)**

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Monitored continuously throughout the study. **Dose-Limiting Toxicities (DLTs):** Strictly monitored during the up to 21-day DLT evaluation period. **Serious Adverse Events (SAEs):** Must be reported within 24 hours to the sponsor and applicable regulatory/ethics boards. **Data Monitoring Committee (DMC):** An independent Data Monitoring Committee will regularly review safety and efficacy data during the Phase 1b/2 trial progression.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations:

- * **Intent-to-Treat (ITT):** All randomized participants (used for efficacy analyses).
- * **Safety Set:** All participants who receive at least one dose of study medication (used for AE reporting).

Sample Size Justification: Approximately 170 participants, powered to detect meaningful safety and early efficacy signals across the multiple investigational and control arms. **Handling of Missing Data:** Standard survival analysis techniques (e.g., Kaplan-Meier estimates for PFS and DOR) with censoring for missing data at the last known adequate assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring visits by the sponsor/CRO to ensure protocol adherence, accurate data entry, and patient safety. **Audit Trail:** Validated Electronic Data Capture (EDC) system with comprehensive audit trails for all queries, corrections, and eCRF locks.

11. Study Identifiers & Governance

Primary ID: MK-6070-003 **Registry Number:** NCT07227597 **Posting ID:** 369951154487414396 **Sponsor/Lead Organization:** Merck Sharp & Dohme (MSD) / Merck **Study Title:** A Clinical Study of Gocatamig (MK-6070) and Infinatamab Deruxtecan (MK-2400) in People With Small Cell Lung Cancer (MK-6070-003) **Official Regulatory Title:** A Phase 1b/2 Open-label Study Evaluating Different MK-6070 and Infinatamab Deruxtecan (MK-2400)-Based Regimens in First-line Extensive Stage Small Cell Lung Cancer

12. Clinical Framework (Oncology Specific)

Primary Condition: Extensive-Stage Small Cell Lung Cancer (ES-SCLC) **Disease Definition:** Small cell lung carcinoma which has spread beyond one hemi-thorax and the regional lymph nodes.

Terminology Codes: MeSH Code: D009369 | SNOMED ID: 708064003

Medical Methodology: Evaluation of T-cell engager (targeting DLL3) + Antibody Drug Conjugate therapies **Optimization Target:** Disease control, management of dose-limiting toxicities, and improvement of PFS over standard of care.

13. Study Procedures & Methodology

Management Pathway: First-line Induction and/or Maintenance therapy **Education Protocol (HCP):** Site Initiation Visits (SIV) and detailed Investigator Brochure training for proper administration of gocatamig and I-DXd. **Education Protocol**

(Patient): Review of informed consent and specific AE monitoring materials (e.g., recognizing signs of Cytokine Release Syndrome or ADC-related toxicities). **Quality Audit Mechanism:** Continuous monitoring of protocol deviations and centralized data reviews.

14. Observation & Follow-Up Schedule

Total Duration: Until disease progression or unacceptable toxicity (estimated through Nov 2030) **Onsite Visit Cadence:** Pre-defined cycles (typically 21-day or 28-day intervals) for IV

infusions and assessments. **Remote Monitoring Frequency:** Between treatment cycles, as clinically indicated. **Checklist Review Interval:** Continuous safety monitoring and periodic scheduled radiographic assessments to evaluate tumor response.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				